

Data Quality, Reconciliation, and Record Correction

Resolving duplicates, discrepancies, stale records, and provenance gaps

Document ID	Version	Effective	Review due
NB-IG-801	1.0	2026-08-01	2027-08-01

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Owner: Information Governance Officer
Approved by: Fictional Clinical Governance Committee
Classification: Training / public synthetic corpus

Document control and RAG metadata

Field	Value
Canonical title	Data Quality, Reconciliation, and Record Correction
Document ID	NB-IG-801
Keywords	data quality, reconciliation, duplicate, correction, provenance, status, patient record
Authority	Information Governance Officer
Supersedes	None - initial synthetic edition
Applicability	Training simulations at Northbridge Cardiology Practice only

Retrieval rule

Retrieve when patient data conflicts, is duplicated, lacks units/timestamps, or appears stale.

1. Data-quality dimensions

Dimension	Check
Identity	Correct patient and encounter; no cross-patient contamination
Completeness	Required dose units, routes, dates, statuses, sources, and owners are present
Consistency	Medication lists, orders, discharge summaries, and patient report do not conflict silently
Timeliness	Clinical values and medicine status are current enough for the intended decision
Provenance	Source system/person, author, timestamp, and version are recorded
Validity	Units, ranges, dates, codes, and status vocabulary are structurally valid

2. Medication reconciliation procedure

- Collect at least two appropriate sources when available: current record, signed orders, pharmacy/discharge list, and patient/carer report.
- For every medicine record name/formulation, strength, dose, unit, route, frequency, indication, start/stop/hold status, last known dose where relevant, source, and certainty.
- Do not merge similar names or formulations automatically.
- Classify discrepancies as intentional/documented, intentional/undocumented, unintentional, or unresolved.
- Unresolved clinically important discrepancies require pharmacist/prescriber review before a final medication plan.

3. Correction process

Step	Action
1	Preserve original record and identify the disputed field
2	Verify against authoritative source and responsible clinician
3	Add corrected value with reason, author, timestamp, and source
4	Update dependent lists and notify relevant teams
5	Re-run affected safety/retrieval checks
6	Close only after reviewer confirms consistency

4. Staleness and missingness

- Display the observation time, not merely the ingestion time.
- Do not call a historical medicine active solely because no end date exists; status and current reconciliation are also required.
- Do not compare numeric results without units and compatible measurement context.
- A missing result is not a normal result.
- A missing allergy record is not proof of no known allergies.
- The final answer must list material missing or stale information and its effect on confidence.

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